



TRACES

FACILITATOR GUIDE

January 2026

TRACEs Assessment

Trauma and Relational Adversity in Caregiver Environments

Facilitator Guide

Thank you for the sacred work you do!

You stand witness to the invisible weight of caregiving. You hold space for stories that the world doesn't want to hear. You see trauma where others see only stress. You name what systems refuse to acknowledge.

This work matters. You matter.

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Section 1: Introduction to TRACEs

1.1 Welcome

Welcome to the TRACEs Assessment facilitator training. As a family peer supporter, you stand in a unique position—you understand the weight of multi-systemic caregiving from lived experience, and you bring that wisdom into spaces where caregivers are often overlooked, misunderstood, or asked to simply 'hold on a little longer.'

This guide will equip you with the knowledge and tools to administer the TRACEs Assessment with confidence, compassion, and clarity. More importantly, it will help you recognize when caregivers are not just stressed—they are trauma-activated—and how to respond with the dignity and support they deserve.

1.2 What is TRACEs?

TRACEs stands for **Trauma and Relational Adversity in Caregiver Environments**. It is a screening tool designed specifically to identify signs of trauma activation in caregivers of multi-system-involved youth.

Unlike general stress assessments, TRACEs recognizes that caregiving in these contexts is not simply challenging—it is trauma-inducing.

Caregivers face:

- Child-to-Parent Violence (physical, verbal, emotional)
- Systemic fragmentation across juvenile justice, mental health, child welfare, and schools
- Financial instability from caregiving-related job loss
- Institutional betrayal—delays, denial of services, gaslighting
- Public stigma and social isolation
- Emotional degradation and moral injury

This is not stress. This is trauma. And yet, it remains unnamed, unmeasured, and unsupported.

1.3 Why TRACEs Matters

"In every high-stakes crisis involving a child, there is someone just out of frame absorbing the chaos, holding the pieces, and often breaking in silence."

ACEs (Adverse Childhood Experiences) transformed how we understand childhood trauma. But caregivers have been left without a parallel lens. TRACEs fills that gap. **When caregivers collapse, systems collapse.** If we don't name caregiver trauma, we can't screen for it, support it, or treat it.

1.4 Why TRACEs Is Not a Strengths-Based Assessment

You may notice that TRACEs focuses on challenges, strain, and areas of difficulty rather than highlighting caregiver strengths or resilience. This is intentional, and it's important to understand why.

TRACEs is explicitly not a strengths-based assessment. Here's why:

Accessibility in Crisis

When caregivers are in crisis or experiencing trauma activation, identifying what is wrong is often more cognitively accessible than articulating what is going well. Asking someone who is drowning to describe their swimming skills is not helpful—they need a lifeline first.

Deficit-focused questions like 'I struggle to manage my child's challenging behaviors' are easier to answer honestly when you're overwhelmed than strengths-focused questions like 'What coping strategies have worked for you?' When you're in survival mode, you don't always have access to that reflective capacity.

Naming Creates Pathways

You cannot address what you cannot name. By explicitly identifying areas of strain—'I feel isolated from my community' or 'Financial concerns are a major source of stress'—caregivers can:

- Identify the weakness/gap - Name what is not working
- Identify options - Explore what could help
- Compare with need - Determine fit and priority
- Develop supports - Build targeted interventions

This process itself builds resilience skills. The ability to accurately assess what you lack is a form of self-awareness that enables help-seeking and problem-solving.

Strengths Don't Erase Trauma

Caregivers of multi-system-involved youth are often told they are 'strong,' 'resilient,' or 'doing such a good job.' While well-intentioned, this can feel invalidating when they feel like they are barely surviving. Strengths-based language can unintentionally minimize suffering and discourage honest disclosure of struggle.

TRACEs says: 'Tell me what is hard. Tell me where you are hurting. I will not ask you to prove you are strong enough to deserve support.'

Building Resilience Through Honest Assessment

True resilience is not pretending everything is fine. It is the capacity to:

- Accurately assess your situation
- Identify what you need
- Ask for help
- Navigate toward support

By helping caregivers name their trauma and identify specific areas of strain, TRACEs equips them to become better advocates for themselves. The assessment becomes a tool for empowerment, not just evaluation.

1.4 Your Role as a Facilitator

As a family peer supporter using TRACEs, you are not a therapist, case manager, or service provider. You are a witness, guide, and advocate.

Your role is to:

- Create a safe space for honest dialogue
- Administer the TRACEs Assessment with compassion and clarity
- Interpret results through a trauma-informed lens
- Connect caregivers to appropriate supports and resources
- Advocate for systemic change based on what you see
- Maintain your own boundaries and practice self-care

1.5 How to Use This Guide

This facilitator guide is organized into ten sections:

1. Introduction to TRACEs (you are here)
2. Understanding Your Role as a Facilitator
3. The TRACEs Framework – deep dive into the six domains
4. Administering the Screener
5. Administering the Full Assessment
6. Interpreting Results and Next Steps
7. Having Difficult Conversations
8. Self-Care and Boundaries
9. Case Studies and Practice Scenarios
10. Resources and Appendices

Each section includes practical guidance, sample scripts, reflection questions, and real-world examples drawn from caregivers like you.

Section 2: Understanding Your Role as a Facilitator

2.1 What Family Peer Support Means in the Context of TRACEs

Family peer support is grounded in shared lived experience. You've walked a similar path. You understand the invisible weight of caregiving for multi-system-involved youth—not from a textbook, but from your own life.

TRACEs doesn't ask you to become a clinician or case manager. It asks you to use your experience to bear witness, to help caregivers see their own trauma clearly, and to guide them toward support without judgment.

2.2 Core Principles for TRACEs Facilitators

Dignity-Centered Practice

Every caregiver you work with deserves to be seen, heard, and valued. This means:

- Never assuming you know their story
- Honoring their agency and choices
- Avoiding savior narratives or fixing language
- Recognizing that you are a guide, not a rescuer

Trauma-Informed Facilitation

Being trauma-informed means understanding that:

- Behavior is communication (defensiveness, withdrawal, anger all have meaning)
- Safety is relational, not just physical
- Choice and control matter deeply
- Caregivers may have been harmed by systems meant to help

Anti-Dependency Orientation

Your goal is not to make caregivers dependent on you, but to empower them toward independence and self-advocacy.

This means:

- Teaching skills, not just providing answers
- Connecting caregivers to resources they can access independently
- Celebrating progress and capacity-building
- Avoiding language that positions you as the only solution

2.3 What You Are NOT Responsible For

It's critical to be clear about boundaries. As a peer supporter using TRACEs, you are NOT responsible for:

- Providing therapy or clinical treatment
- Making service eligibility determinations
- "Fixing" the caregiver's situation

- Managing crises beyond your scope (e.g., active suicidality, imminent danger)
- Carrying the emotional weight of their trauma

Your role is to open pathways, not to carry people through them.

2.4 When to Refer Out

There will be moments when a caregiver needs more support than peer facilitation can provide. Refer to clinical or crisis services when:

- The caregiver expresses active suicidal ideation or intent
- There are immediate safety concerns (active violence, child abuse/neglect)
- The caregiver is experiencing acute mental health crisis (psychosis, severe dissociation)
- Substance use is creating dangerous conditions
- The situation requires legal intervention or child protective services

Warm handoffs are essential. Don't just give a phone number—help the caregiver make the call, stay with them if appropriate, and follow up to ensure connection.

2.5 Reflection Questions

Before you begin facilitating TRACES, take time to reflect:

- What draws you to this work?
- What parts of your own caregiving story might show up when working with others?
- Where do you need support or supervision?
- What boundaries do you need to protect your own well-being?
- How will you know when you need to step back or refer out?

Section 3: The TRACEs Framework

3.1 Overview: The Six Domains

TRACEs is built around six interconnected domains that capture the full scope of caregiver trauma. Understanding these domains deeply will help you recognize patterns, ask better questions, and guide caregivers toward meaningful support. Each domain explores how trauma accumulates—not just from one event, but from chronic exposure to relational rupture, systemic failure, and moral injury.

The Six Domains:

1. Parent-to-Child Impact
2. Spousal/Nuclear Family Impact
3. Extended Family Impact
4. Social/Community Impact
5. Financial Strain
6. Household Safety

3.2 Domain 1: Parent-to-Child Impact

What This Domain Captures:

This domain focuses on the caregiver's direct relationship with their child. It measures emotional exhaustion, feelings of inadequacy, and the strain on the parent-child bond.

Key Indicators:

- Difficulty understanding the child's needs
- Struggling to manage challenging behaviors
- Loss of meaningful communication
- Feeling overwhelmed and under-resourced
- Guilt and shame over "not being enough"
- Strained emotional bond

Why This Matters:

When the parent-child relationship suffers, it affects everything. Caregivers often internalize blame, believing they are failing their child. This domain helps name that the relationship strain is a symptom of systemic failure, not personal inadequacy.

Sample Questions to Explore:

- "When was the last time you felt connected to your child?"
- "What do you think your child needs most right now?"
- "What support would help you feel more equipped as a parent?"

3.3 Domain 2: Spousal/Nuclear Family Impact

What This Domain Captures:

This domain examines how caregiving stress affects intimate partnerships and the nuclear family unit. It recognizes that trauma ripples outward, straining marriages, co-parenting relationships, and sibling dynamics.

Key Indicators:

- Conflicts over parenting disagreements
- Clashing parenting styles
- Loss of quality family time
- Feeling unsupported by partner
- Relationship strain affecting overall family happiness
- Physical or mental health decline due to parenting stress

Why This Matters:

Partnerships often become casualties of caregiving trauma. This domain validates that relationship strain is not about personal failure—it's about being under relentless pressure without adequate support.

Sample Questions to Explore:

- "How is your partner handling all of this?"
- "Do you and your partner have time to connect outside of crisis management?"
- "What would help you feel more aligned as a parenting team?"

3.4 Domain 3: Extended Family Impact

What This Domain Captures:

This domain focuses on relationships with grandparents, siblings, in-laws, and broader family networks. It measures judgment, lack of understanding, and the loss of traditional family support systems.

Key Indicators:

- Extended family unsupportive of parenting approach
- Child's behavior creates tension at family gatherings
- Lack of practical help with childcare or daily needs
- Feeling judged or misunderstood
- Extended family gatherings increase stress rather than provide respite

Why This Matters:

Extended family can be a lifeline—or another source of trauma. When grandparents dismiss

diagnoses, blame the caregiver, or refuse to accommodate the child's needs, it deepens isolation and shame.

Sample Questions to Explore:

- "Does your extended family understand what you're dealing with?"
- "Do family gatherings feel supportive or stressful?"
- "What would you need from extended family to feel less alone?"

3.5 Domain 4: Social/Community Impact

What This Domain Captures:

This domain explores how caregivers interact with schools, doctors' offices, community programs, and public spaces. It measures isolation, stigma, and the exhausting work of advocating for a child who doesn't fit neatly into existing systems.

Key Indicators:

- Feeling isolated from community
- Schools difficult to work with
- Limited social activities due to child's needs
- Feeling judged by others
- Advocacy work feels overwhelming
- Community programs don't accommodate child's needs

Why This Matters:

Schools, doctors' offices, and community programs often fail caregivers—not through malice, but through lack of training, resources, or flexibility. This domain names that failure and validates the exhaustion of constant advocacy.

Sample Questions to Explore:

- "How are things going with your child's school?"
- "Do you have people in your community who understand what you're going through?"
- "What would make public spaces feel safer or more welcoming for your family?"

3.6 Domain 5: Financial Strain

What This Domain Captures:

This domain measures the financial burden of caregiving—from medical bills and therapy costs to lost income from job changes or leaving the workforce entirely.

Key Indicators:

- Significant financial burden from child's needs
- Difficulty affording resources and support

- Made career or job changes due to child's needs
- Financial concerns a major source of family stress
- Cut back on non-essential expenses
- Uncertainty about long-term financial stability

Why This Matters:

Financial strain compounds all other trauma. When caregivers can't pay bills, afford therapy, or maintain employment, the stress becomes existential. This domain helps make the invisible visible.

Sample Questions to Explore:

- "Has caregiving affected your ability to work?"
- "Are there supports or services you can't access because of cost?"
- "What would financial stability look like for your family right now?"

3.7 Domain 6: Household Safety

What This Domain Captures:

This is the most urgent domain. It measures physical safety concerns, risk of harm, and the chronic hypervigilance that comes from living in an unpredictable or dangerous environment.

Key Indicators:

- Concerns about household safety
- Child threatens or is physically violent
- Unprepared for emergencies or safety concerns
- Worried about safety of other children in home
- Home lacks necessary safety equipment or modifications
- Emergency situations common and poorly managed

Why This Matters:

Safety concerns override everything. Any 'Yes' responses in this domain require immediate attention and safety planning. This domain also captures the invisible toll of living in chronic threat—the hypervigilance, the interrupted sleep, the constant scanning for danger.

Sample Questions to Explore:

- "Do you feel safe in your home right now?"
- "What happens when your child becomes aggressive?"
- "What would help you feel more prepared for emergencies?"

Section 4: Administering the Screener

4.1 When to Use the Screener

The TRACEs Screener is a 12-question screening tool using Yes/No/Not Applicable responses. It's designed for situations where time is limited or when you need a quick snapshot of caregiver strain.

Use the Screener when:

- You have 10-15 minutes for assessment
- The caregiver is new to services or unfamiliar with assessment tools
- You're in a team meeting and need a rapid check-in
- You want to gauge whether deeper assessment is needed
- The caregiver is emotionally fragile and can't handle a longer assessment

4.2 Preparing the Environment

Before administering the Screener:

- Find a private, quiet space – no distractions, interruptions, or observers
- Set aside 20-30 minutes – this includes time for introduction and debrief
- Have resources ready – crisis numbers, referral options, grounding techniques
- Check your own emotional state – if you're depleted, reschedule

4.3 Introduction Script

Use language like this to introduce the Screener:

"I'd like to walk through a brief assessment with you. It's called TRACEs, and it helps us understand how caregiving for your child is affecting different parts of your life—your relationship with your child, your family, your community, finances, and safety. There are 12 questions, and you'll answer Yes, No, or Not Applicable for each one. This isn't about judging you. It's about seeing clearly where you might need support. Does that sound okay?"

Key points to emphasize:

- This is not an eligibility test
- There are no "right" answers
- Honesty helps us connect them to the right support
- They can skip questions or stop at any time

4.4 Administering the Questions

Read each question slowly and clearly. Allow time for the caregiver to process before responding.

The 12 Questions:

- 1.(PCI) I find it challenging to understand and respond to my child's needs.
- 2.(PCI) I struggle to manage my child's challenging behaviors effectively.
- 3.(S/NFI) Conflicts due to parenting disagreements frequently arise in our family.
- 4.(S/NFI) Changes and challenges disrupt our family routines significantly.
- 5.(EFI) Involvement with extended family often leads to more stress rather than well-being.
- 6.(EFI) Extended family lacks understanding and acceptance of my child's needs.
- 7.(S/CI) I often feel isolated from my community due to my child's needs or behaviors.
- 8.(S/CI) Advocating for my child's needs in social and community settings is overwhelming.
- 9.(FS) Caring for my child's needs places a significant financial burden on our family.
- 10.(FS) I have had to make difficult career or job changes due to my child's needs.
- 11.(HS) Our home lacks necessary equipment or modifications for my child's safety needs.
- 12.(HS) My child threatens or is physically violent with me or others in my home.

Tips for facilitation:

- Pause after each question to allow processing
- If the caregiver asks for clarification, reframe in their language
- Validate responses without inserting your own opinion
- Track "Yes" responses—you'll need the count for scoring

4.5 Understanding "Not Applicable"

The "Not Applicable" option is crucial. Not all caregivers have a spouse, extended family, or employment to lose. "Not Applicable" allows them to skip questions that don't fit their situation without feeling like they're failing to answer.

Examples:

- Single parent with no extended family contact → "Not Applicable" for extended family questions
- Single-parent who is not employed → "Not Applicable" for job-related questions
- Only child in household → "Not Applicable" for sibling safety questions

4.6 Scoring the Screener

Count the total number of "Yes" responses. The score determines the trauma activation level and suggested next steps.

# of "Yes"	Activation Level	Interpretation	Suggested Next Steps
0–2	Stable with Strain	Minor challenges. No clear trauma activation.	Validate experience, offer self-guided tools, monitor over time.
3–5	Systemic Tension	Early signs of stress accumulation. Patterns forming.	Open dialogue, lightweight care plan, peer support.
6–9	Trauma Activation Present	Relational, financial, or systemic trauma affecting functioning	Begin care plan, team huddle, cross-system coordination.
10–12	Crisis Embedded in Care	Signs of moral injury, deep fatigue, household instability.	Escalate planning, multidisciplinary review, wraparound supports.

SAFETY FLAG PROTOCOL: If either safety question (#11 or #12) is marked "Yes," initiate immediate safety conversation regardless of total score.

4.7 Debrief and Next Steps

After scoring, share the results with the caregiver. Use clear, non-clinical language.

Sample debrief script:

"Based on your responses, I'm seeing [activation level]. That means [interpretation]. What this tells me is that you're dealing with [specific challenges]. The next step I'd recommend is [suggested action]. How does that sound to you?"

Always end with validation: "What you're experiencing is real, and it makes sense given everything you're managing. You deserve support."

Section 5: Administering the Full Assessment

5.1 When to Use the Full Assessment

The Full TRACEs Assessment contains 72 questions across six domains using a 0–4 Likert scale. This comprehensive tool provides detailed insight into the degree and location of caregiver trauma.

Use the Full Assessment when:

- You have 45–60 minutes for comprehensive assessment
- The Short-Form indicates significant trauma activation
- You're developing a detailed care plan or service coordination strategy
- You need to measure depth of progress over time
- The caregiver wants a deeper exploration of their experience

5.2 Understanding the Likert Scale

The Full Assessment uses a 0–4 scale where caregivers rate how accurately each statement reflects their current situation:

- 0 (0%) – Does not reflect their experience at all; issue completely absent
- 1 (25%) – Reflects experience minimally; occurs rarely with little impact
- 2 (50%) – Somewhat reflects experience; occurs occasionally with moderate impact
- 3 (75%) – Reflects experience quite well; frequent with significant impact
- 4 (100%) – Completely reflects experience; persistent, severe, major impact

5.3 Administration Guidelines

Before You Begin:

- Ensure you have 60–90 minutes including debrief time
- Have water, tissues, and comfort items available
- Let the caregiver know they can take breaks
- Remind them there are no wrong answers

Pacing the Assessment:

- Move through questions at the caregiver's pace, not yours
- Watch for signs of overwhelm (tearfulness, shutting down, agitation)
- Offer breaks between domains
- Validate experiences as they share, *but don't over-process during administration*

5.4 Scoring the Full Assessment

Each domain has 12 questions, each scored 0–4. Calculate totals for each domain separately, then add for the overall score.

Domain Scoring:

- Each domain: 0-48 points possible (12 questions × 4 max points)
- Total score: 0-288 points possible (6 domains × 48 points)

# of "Yes"	Activation Level	Interpretation	Suggested Next Steps
0-72	Sustained Stability	Few stress indicators. Caregiver feels equipped.	Low need – monthly or biweekly check-ins if requested
73-144	Emerging Disruption	Moderate strain across multiple domains.	Moderate need – biweekly sessions, care mapping
145-216	Trauma Activation Present	Caregiver fatigue, exhaustion, systemic distrust impairing well-being	High need – weekly sessions, coordinated supports, advocacy
217-288	Fracture Point/Systemic Harm	Critical trauma patterns. Risk of caregiver collapse or household instability.	Severe need – weekly plus additional check-ins, intensive wraparound

Section 6: Interpreting Results and Next Steps

6.1 The Art of Interpretation

Interpreting TRACeS results is not mechanical. Numbers tell part of the story, but you must also read between the lines—noticing what was said, what wasn't said, body language, emotion, and context.

Listen for:

- Minimization: "It's not that bad" when scores suggest otherwise
- Shame: Apologizing for responses, self-blame
- Resignation: "Nothing ever changes anyway"
- Isolation: "No one understands"
- Distrust: Hesitation to share, skepticism about support

6.2 Sharing Results with Caregivers

When you debrief the assessment, lead with validation, not numbers.

Sample Debrief Script:

"Thank you for being so honest with me. What I'm seeing is [brief summary of highest-strain domains]. The overall picture shows [activation level]. What that means is [interpretation in plain language]. This isn't about you failing—this is about a system that hasn't given you what you need. Based on what you've shared, here's what I think might help: [specific next steps]. What feels most important to you right now?"

Key Principles:

- Name the trauma, don't pathologize the person
- Connect strain to systems, not personal inadequacy
- Offer choices, not directives
- Ask what feels most urgent to them

6.3 Creating Action Plans

Based on TRACeS results, work with the caregiver to identify 2–3 priority areas. Don't overwhelm them with a 15–point plan.

Action Plan Template:

- Priority Domain: [What domain showed highest strain?]
- Immediate Action: [What can happen this week?]
- Short-term Goal (1–3 months): [What would reduce strain?]
- Resources/Referrals: [Who can help?]
- Follow-up: [When will we check in again?]

6.4 Common Referral Pathways

Based on TRACEs results, you may need to connect caregivers to:

- Crisis Services: Suicide hotline, mobile crisis, domestic violence hotline
- Clinical Support: Individual therapy, family therapy, psychiatric services
- Case Management: Wraparound coordination, care management
- Advocacy: IEP advocates, legal aid, disability rights organizations
- Financial Assistance: Benefits navigation, emergency funds, food assistance
- Respite: In-home respite, crisis respite, short-term placement
- Peer Support: Support groups, family-to-family programs, peer mentorship

6.5 Documenting and Tracking Progress

Keep clear documentation of TRACEs scores, action plans, and referrals made. This helps track progress and demonstrates need for services.

Document:

- Date and context of assessment
- Overall score and domain breakdown
- Safety concerns flagged
- Action plan agreed upon
- Referrals made and connection status
- Follow-up date scheduled

Section 7: Having Difficult Conversations

7.1 When Conversations Get Hard

TRACEs will surface difficult truths: violence in the home, suicidal ideation, social isolation, financial devastation. You cannot shy away from these conversations, but you must enter them with skill and care.

Core Principles:

- Don't avoid; lean in gently
- Name what you're observing without judgment
- Center the caregiver's safety and agency
- Know your mandated reporting responsibilities

7.2 Addressing Child-to-Parent Violence

When a caregiver discloses that their child is physically or verbally violent toward them, many feel deep shame. They may minimize, make excuses, or blame themselves.

How to Respond:

"I'm hearing that your child has been physically aggressive with you. That must be incredibly hard—and it's not something you should have to manage alone. Can you tell me more about what happens? How do you stay safe when it escalates?"

Validate:

- "This is real violence, even if it's from your child"
- "You deserve to be safe in your own home"
- "This isn't your fault—it's a sign that everyone needs more support"

Next Steps:

- Safety planning: de-escalation strategies, safe spaces, emergency contacts
- Crisis intervention services if violence is escalating
- Family-based crisis intervention or mobile crisis team
- Therapeutic support for both caregiver and child

7.3 When You Suspect Suicidal Ideation

If a caregiver expresses hopelessness, exhaustion that feels unbearable, or hints at not wanting to be here anymore, ask directly.

How to Ask:

"I'm hearing how exhausted you are, and I want to ask you something directly: Have you been

thinking about hurting yourself or not wanting to be here anymore?"

Asking does not plant the idea—it opens the door to help.

If the answer is yes:

- Ask about intent: 'Do you have a plan?'
- Ask about timeline: 'When were you thinking about this?'
- Don't leave them alone if risk is immediate
- Call 988 Suicide & Crisis Lifeline together or help them access mobile crisis
- Follow up within 24 hours

Always follow your organizational procedure for suicidal ideation.

7.4 Mandated Reporting: What You Need to Know

As a family peer supporter, you may be a mandated reporter depending on your state and organizational role. Know your legal obligations.

If you are a mandated reporter, you MUST report if you have reasonable suspicion of:

- Child abuse or neglect
- Imminent harm to a child
- Elder or dependent adult abuse (in some states)

How to Navigate Reporting:

- Be transparent: 'Based on what you've shared, I'm legally required to make a report'
- Explain the process: 'I'll call Children's Services, and here's what will happen next'
- Offer to stay with them during or after the call
- Don't disappear—maintain connection even after the report

7.5 When the System Has Failed Them

Sometimes caregivers will share stories of profound institutional betrayal: therapists who gave up, schools that expelled their child, doctors who dismissed their concerns, agencies that closed cases without resolution.

How to Respond:

"That should never have happened. You deserved better. The system failed you, and that is not okay. What would you need to feel like someone is finally listening?"

Don't defend the system. Validate the harm.

Section 8: Self-Care and Boundaries

8.1 Why Self-Care Isn't Optional

You cannot pour from an empty cup. As a peer supporter holding space for caregiver trauma, you will absorb pain, witness injustice, and carry stories home with you. If you don't tend to your own well-being, you will burn out—and that helps no one.

Self-care is not selfish. It's sustainable practice.

8.2 Recognizing Vicarious Trauma

Vicarious trauma occurs when you take on the emotional weight of others' experiences. It's different from burnout—it changes how you see the world.

Signs of Vicarious Trauma:

- Intrusive thoughts about caregivers' stories
- Emotional numbness or cynicism
- Difficulty sleeping or nightmares
- Hypervigilance about your own children's safety
- Avoiding this work or dreading sessions
- Loss of hope or belief in change

8.3 Setting Healthy Boundaries

Boundaries protect both you and the caregivers you support. They're not walls—they're guidelines for sustainable, respectful relationships.

Essential Boundaries:

- Time: Set clear session lengths and availability hours. Don't answer texts at midnight.
- Scope: Be clear about what you can and can't provide. Refer out when needed.
- Emotional: You can care deeply without taking personal responsibility for outcomes.
- Personal disclosure: Share your story strategically, not reactively.

8.4 Practical Self-Care Strategies

Daily Practices:

- Grounding exercises after sessions (deep breathing, body scan, movement)
- Clear transition rituals between work and home
- Journal or debrief with a trusted colleague

Weekly Practices:

- Supervision or peer consultation

- Activities that bring you joy unrelated to this work
- Time in nature, creative expression, connection with loved ones

Monthly Practices:

- Reflect on your own trauma history and triggers
- Assess your caseload and capacity
- Celebrate wins, even small ones

8.5 When to Step Back

Sometimes the most responsible thing you can do is step back. This doesn't mean you're failing—it means you're being honest about your capacity.

Step back if:

- You're experiencing signs of vicarious trauma that aren't improving
- Your own mental health is declining
- You're violating boundaries or becoming enmeshed
- You're losing objectivity or projecting your own story
- You're consistently exhausted, resentful, or dreading this work

Section 9: Case Studies and Practice Scenarios

9.1 Case Study: Tanya - Foster Parent

Background:

Tanya has been fostering Malik (11) and Ava (7) for eight months. From the outside, her life looks ideal—steady middle-school teaching job, supportive partner, spare bedrooms, and years of foster-care training. Inside the house, though, things are unraveling. Malik has begun punching walls hard enough to leave dents, and Ava hoards food under her bed. The caseworkers keep asking her to 'hang on a little longer,' promising services that never materialize. Her partner has grown resentful of the constant chaos, and nightly arguments erupt over safety and schedules. Everyone keeps telling her how lucky the kids are to have her. No one asks what it's costing her to be the stable adult holding the whole structure upright.

TRACEs Assessment Results:

- Short-Form: 8 Yes responses (Trauma Activation Present)
- Safety concerns flagged (property damage, escalating behaviors)
- Highest strain: Parent-to-Child, Spousal/Nuclear Family, Social/Community\

Discussion Questions:

- What would your immediate response be to Tanya?
- Which domain would you prioritize first, and why?
- What referrals would you make?
- How would you address the systemic failure (delayed services, false promises)?

9.2 Practice Scenario: Marcus - Single Father

Situation:

Marcus is a single father of 13-year-old Jordan, who was recently placed on probation for shoplifting. Jordan has ADHD and struggles in school. Marcus works two jobs to make ends meet and rarely sees Jordan awake. He's exhausted, disconnected from his son, and worried about what happens next. He scored 5 Yes responses on the Short-Form, with highest concern in Financial Strain and Parent-to-Child domains.

Discussion Questions:

- How would you validate Marcus's experience?
- What immediate support would you suggest?
- How would you help him reconnect with Jordan despite time constraints?

9.3 Practice Scenario: Elena - Grandmother Raising Grandchildren

Situation:

Elena (62) is raising her two grandchildren (ages 6 and 9) after her daughter's incarceration. Both children have experienced trauma and have significant behavioral needs. Elena is on a fixed income, managing her own health conditions, and feels completely isolated. Her extended family blames her for her daughter's choices. She scored 10 Yes responses (Crisis Embedded in Care), with critical scores in Extended Family, Financial, and Household Safety domains.

Discussion Questions:

- How do you address the urgent safety and financial needs?
- What support specific to kinship caregivers would you connect her to?
- How would you validate her experience of blame and isolation?

9.4 Practice Scenario: Safety Domain Override

Situation:

During a Full Assessment, a caregiver scores relatively low overall (85 total) but gives a score of 4 to the statement: 'My child threatens or is physically violent with me or others in my home.' When you ask about it, she says her 15-year-old son has punched holes in walls, thrown objects, and once shoved her into a doorframe. She minimizes it: 'He doesn't mean it. He's just frustrated. I can handle it.'

Discussion Questions:

- What do you say to her immediately?
- How do you activate the Safety Domain Override?
- What safety planning do you initiate?
- What if she resists help?

9.5 Practice Scenario: Institutional Betrayal

Situation:

A caregiver shares that their child's therapist terminated services after three sessions because 'he's not making progress.' The school expelled their child for behaviors related to their disability. Child Welfare closed their case without providing any of the promised supports. The caregiver is angry, distrustful, and on the verge of giving up entirely. They say: 'Why should I believe you'll be any different?'

Discussion Questions:

- How do you acknowledge the harm?
- What do you say about why you're different (or not)?
- How do you rebuild trust?

Section 10: Resources and Appendices

10.1 Crisis Resources

National Crisis Lines:

- 988 Suicide & Crisis Lifeline: Call or text 988
- National Domestic Violence Hotline: 1-800-799-7233
- Childhelp National Child Abuse Hotline: 1-800-422-4453
- Crisis Text Line: Text HOME to 741741
- SAMHSA National Helpline: 1-800-662-4357 (mental health/substance use)

10.2 Documentation Templates

Suggested TRACeS Assessment Documentation:

- Date of assessment:
- Assessment type: Short-Form / Full
- Overall score:
- Domain breakdown:
- Safety concerns flagged: Yes / No
- Activation level:
- Priority domains identified:
- Action plan:
- Referrals made:
- Follow-up date:

10.3 Sample Safety Plan

Suggested Household Safety Plan Template:

- Triggers identified: [What situations escalate to violence?]
- De-escalation strategies: [What helps calm the situation?]
- Safe spaces: [Where can caregiver/other children go if needed?]
- Emergency contacts: [Who to call: mobile crisis, trusted friend, 911]
- Code words: [Safe way to signal for help]
- Follow-up plan: [When will we review this?]

10.4 Additional Reading

- *The Body Keeps the Score* by Bessel van der Kolk
- *Trauma Stewardship* by Laura van Dernoot Lipsky
- *The Deepest Well* by Nadine Burke Harris
- *What Happened to You?* by Bruce Perry and Oprah Winfrey
- *The Myth of Normal* by Gabor Maté

